

SUPPLEMENTAL MATERIAL FOR:

Apalutamide in Patients With Metastatic Castration-Sensitive Prostate Cancer: Final Survival
Analysis of the Randomized, Double-Blind Phase III TITAN Study

Kim N. Chi, Simon Chowdhury, Anders Bjartell, Byung Ha Chung, Andrea J. Pereira de Santana
Gomes, Robert Given, Alvaro Juárez Soto, Axel S. Merseburger, Mustafa Özgüroğlu, Hirotsugu
Uemura, Dingwei Ye, Sabine Brookman-May, Suneel D. Mundle, Sharon A. McCarthy, Julie S.
Larsen, Weili Sun, Katherine B. Bevens, Ke Zhang, Nibedita Bandyopadhyay, and Neeraj Agarwal

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Supplemental Methods

Study Design and Conduct

Briefly, a broad population of patients with mCSPC were randomized 1:1 to receive apalutamide (240 mg daily) or matched placebo orally once daily in addition to continuous ADT. The first patient was enrolled on December 15, 2015, and the last on July 25, 2017. Prior treatment for mCSPC was limited to previous docetaxel (up to 6 cycles, with the last dose $\leq$ 2 months prior to randomization and with no evidence of progression during treatment or before randomization), ADT for $\leq$ 6 months, and 1 course of radiation or surgical intervention completed prior to randomization. Allowed prior therapies for localized prostate cancer, including $\leq$ 3 years of ADT and other localized treatments (radiation, prostatectomy, lymph node dissection, or systemic therapies), had to be completed at least 1 year before randomization. Patients were stratified by Gleason score at diagnosis ($\leq$ 7 v $>$ 7), region (North America and European Union v all other countries), and prior treatment with docetaxel (yes v no). Review boards at all participating institutions approved the trial, which was conducted in accordance with current International Conference on Harmonisation guidelines for Good Clinical Practice and according to principles set forth in the Declaration of Helsinki. All patients provided written informed consent.

Definitions of Secondary and Other Clinically Relevant End Points

- Time to initiation of cytotoxic chemotherapy was defined as time from randomization to initiation of cytotoxic chemotherapy
- Time to pain progression was defined as the time from the date of randomization to the date of the first observation of pain progression
- Time to chronic opioid use was defined as time from randomization to chronic opioid use

- Time to skeletal-related events was defined as time from randomization to date of the occurrence of symptomatic skeletal event (ie, pathologic fracture, spinal cord compression, radiation to bone, or surgery to bone)
- Time to symptomatic local progression was defined as time from randomization to first occurrence of urethral obstruction or bladder outlet obstruction
- Time to PSA progression was defined as time from randomization to date of PSA progression based on Prostate Cancer Clinical Trials Working Group 2 (PCWG2) criteria¹
- Second progression-free survival was defined as time from randomization to the first occurrence of investigator-determined disease progression (PSA progression, progression on imaging, or clinical progression) on first subsequent therapy or death
- Time to castration resistance was defined as time from randomization to radiographic disease progression, PSA progression per PCWG2 criteria,¹ or symptomatic skeletal event, whichever occurred first.

Sample Size Consideration for Interim and Final Analyses

A total of 410 events were required to detect an HR of 0.75 for the control group at a 2-tailed significance level of 0.045 for the preplanned final OS analysis. The first interim analysis has been performed at $\approx 50\%$ (or ≈ 205) of total required events;² the second interim analysis was planned at $\approx 70\%$ (or ≈ 287) of total required events. The second interim analysis was not performed.

Statistical Analysis of Secondary End Points at the First Interim Analysis

At the first interim analysis,² evaluation of secondary end points was performed in a hierarchical order, each with an overall 2-sided significance level of 0.05: time to cytotoxic chemotherapy, time to pain progression, time to chronic opioid use, and time to skeletal-related event. The time to cytotoxic chemotherapy had a statistically significant between-group difference at that time.² The

time to pain progression did not reach between-group statistical significance; therefore, no formal testing for further secondary end points was conducted.²

Inverse Probability Censoring Weighted Log-Rank Test

The inverse probability of censoring weighting (IPCW) analysis is described in the TITAN Statistical Analysis Plan. In the event that a large number of patients in the placebo group crossed over to apalutamide, a preplanned exploratory analysis using the IPCW log-rank test as described by Cole and Hernan³ was performed to estimate the true treatment effect on overall survival (OS). According to the IPCW method, patients who crossed over from placebo to apalutamide were censored at the time of crossover, while patients remaining in the placebo group were weighted to compensate for missing data. The bias introduced by this informative crossover was corrected by weighting each patient by the inverse of his predicted probability of not being censored at a given time. The first step in the IPCW analysis was to predict the probability of crossover on the basis of each patient's baseline characteristics by fitting a logistic regression model. Finally, OS was analyzed with the censored data set and observations weighted by the inverse of the predicted probability of censoring.

Censoring Rules

Patients who were lost to follow-up or withdrew from the study, those without evidence of radiographic progressive disease, and those who died are censored on the date of their last tumor assessment. Patients who received selected new systemic anticancer therapy without documented disease progression are censored on the date of their last tumor assessment prior to the start of the new systemic therapy or death. Patients with 2 or more consecutive missing assessments followed by evidence of radiographic progressive disease or death are censored on the date of the last tumor

assessment before the missing assessments. Patients who had no tumor assessment at baseline or no tumor assessment after baseline are censored on the date of randomization.

Supplemental Results

Safety

Grade 3 to 4 treatment-emergent adverse events (TEAEs) were reported in 49.4%, 41.7%, and 27.4%, and serious TEAEs were reported in 29.2%, 21.8%, and 13.9% of apalutamide-treated, placebo-treated, and crossover patients, respectively. TEAEs led to treatment discontinuation in 62 (11.8%) patients receiving apalutamide plus ADT, 30 (5.7%) patients receiving placebo plus ADT, and 16 (7.7%) in crossed-over patients.

Supplemental Table S3 summarizes incidences of the most common TEAEs and TEAEs of interest by group terms and grade. Skin rash of any grade was more common in the apalutamide group than in the placebo group (29.2% v 9.3%, respectively). Most skin rash events were grade 1 to 2.

Fractures and falls were reported in 10.3% and 9.4% of patients in the apalutamide group and in 4.9% and 7.0% of patients in the placebo group, respectively; most fractures and falls were grade 1 to 2. Ischemic heart disease (5.9% in the apalutamide group, 2.1% in the placebo group) and cerebrovascular disorders (2.5% and 1.5%, respectively) of any grade were infrequent, most of them of grade 1 to 2. Three patients in the apalutamide group and 2 in the placebo group reported grade 5 ischemic heart disease events. Two patients in the apalutamide group and 3 in the crossover group reported grade 5 ischemic cerebrovascular disorder events.

Supplemental References

1. Scher HI, Halabi S, Tannock I, et al: Design and end points of clinical trials for patients with progressive prostate cancer and castrate levels of testosterone: recommendations of the Prostate Cancer Clinical Trials Working Group. *J Clin Oncol* 26:1148-1159, 2008
2. Chi KN, Agarwal N, Bjartell A, et al: Apalutamide for metastatic, castration-sensitive prostate cancer. *N Engl J Med* 381:13-24, 2019
3. Cole SR, Hernan MA: Adjusted survival curves with inverse probability weights. *Comput Methods Programs Biomed* 75:45-49, 2004

SUPPLEMENTAL TABLE S1. Demographics and Baseline Disease Characteristics^a

	Apalutamide + ADT (n = 525)	Placebo + ADT (n = 527)
Median age (range), years	69 (45 to 94)	68 (43 to 90)
Gleason score at initial diagnosis, n (%) ^b		
< 7	41 (7.8)	39 (7.4)
7	133 (25.3)	130 (24.7)
> 7	351 (66.9)	358 (67.9)
ECOG PS score, n (%) ^c		
0	328 (62.5)	348 (66.0)
1	197 (37.5)	178 (33.8)
2	0	1 (0.2)
Disease volume, n (%)		
Low	200 (38.1)	192 (36.4)
High	325 (61.9)	335 (63.6)
Previous docetaxel treatment, n (%) ^d	58 (11.0)	55 (10.4)
Previous therapy for localized disease, n (%) ^e	94 (17.9)	79 (15.0)
Median PSA (range), µg/mL	5.97 (0 to 2682)	4.02 (0 to 2229)
Metastatic stage at initial diagnosis, n (%)		
M0	85 (16.2)	59 (11.2)
M1	411 (78.3)	441 (83.7)
MX	29 (5.5)	27 (5.1)
Mean baseline BPI-SF pain score, n (%)		
0: no pain	198 (37.7)	200 (38.0)
1 to 3: mild pain	195 (37.1)	207 (39.3)
3 to 7: moderate pain	98 (18.7)	95 (18.0)
8 to 10: severe pain	12 (2.3)	11 (2.1)
Missing data	22 (4.2)	14 (2.7)

^aBetween-group differences were not evaluated statistically, but there were no substantial differences between the 2 groups. Percentages may not total 100 because of rounding.

^bScores on the Gleason scale range from 2 to 10, with higher scores indicating higher-grade cancer that may be more aggressive.

^cEastern Cooperative Oncology Group performance status (ECOG PS) scores range from 0 to 5, with higher scores reflecting greater disability.

^dOf the patients with previous docetaxel use, 27 (47%) in the apalutamide group and 22 (40%) in the placebo group had a node stage of N1 at diagnosis.

^ePrevious therapies for localized prostate cancer included prostatectomy and radiotherapy.

Abbreviations: ADT, androgen deprivation therapy; BPI-SF, Brief Pain Inventory-Short Form; PSA, prostate-specific antigen.

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SUPPLEMENTAL TABLE S2. All First Subsequent Systemic Therapy for Prostate Cancer.

	Apalutamide + ADT (n = 525)	Placebo + ADT (n = 527)
Patients alive at treatment discontinuation (denominator for the table below)	247	345
Patients with first subsequent systemic therapy	120 (48.6)	221 (64.1)
Hormonal	58 (23.5)	124 (35.9)
Abiraterone acetate plus prednisone	27 (10.9)	65 (18.8)
Bicalutamide	16 (6.5)	30 (8.7)
Enzalutamide	9 (3.6)	24 (7.0)
Diethylstilbestrol	2 (0.8)	0
Flutamide	2 (0.8)	4 (1.2)
Goserelin	1 (0.4)	0
Leuprorelin	1 (0.4)	1 (0.3)
Cyproterone	0	2 (0.6)
Octreotide	0	1 (0.3)
Triptorelin	0	1 (0.3)
Chemotherapy	50 (20.2)	88 (25.5)
Docetaxel	42 (17.0)	78 (22.6)
Carboplatin	4 (1.6)	4 (1.2)
Cabazitaxel	2 (0.8)	5 (1.4)
Etoposide	2 (0.8)	3 (0.9)
Paclitaxel	2 (0.8)	1 (0.3)
Estramustine	1 (0.4)	1 (0.3)
Vinflunine	1 (0.4)	0
Cisplatin	0	2 (0.6)
Cyclophosphamide	0	1 (0.3)
Lobaplatin	0	1 (0.3)
Other	32 (13.0)	63 (18.3)
Prednisone	7 (2.8)	20 (5.8)

Prednisolone	6 (2.4)	12 (3.5)
Zoledronic	6 (2.4)	14 (4.1)
Radium-223	5 (2.0)	5 (1.4)
Dexamethasone	4 (1.6)	4 (1.2)
Sipuleucel-t	4 (1.6)	5 (1.4)
Clodronate	1 (0.4)	1 (0.3)
Masitinib	1 (0.4)	0
Niraparib	1 (0.4)	0
Investigational	0	1 (0.3)
Investigational	0	1 (0.3)
Ketoconazole	0	1 (0.3)
Meprednisone	0	1 (0.3)
Methylprednisolone	0	1 (0.3)
Nivolumab	0	1 (0.3)
Olaparib	0	1 (0.3)
Ondansetron	0	1 (0.3)
Pheniramine	0	1 (0.3)
Poly	0	1 (0.3)
Ranitidine	0	1 (0.3)

SUPPLEMENTAL TABLE S3. Incidence of Treatment-Emergent Adverse Events in the TITAN Safety Population (N = 1,051)

Adverse event, n (%) ^a	Apalutamide + ADT (n = 524)	Placebo + ADT (n = 527)	Crossover (placebo to apalutamide) + ADT (n = 208)
Patients with ≥ 1 TEAE ^b	510 (97.3)	510 (96.8)	174 (83.7)
Patients with ≥ 1 grade 3 to 4 TEAE	259 (49.4)	220 (41.7)	57 (27.4)
Any serious TEAE ^b	153 (29.2)	115 (21.8)	29 (13.9)
Any TEAE leading to treatment discontinuation	62 (11.8)	30 (5.7)	16 (7.7)
TEAE leading to death	20 (3.8)	17 (3.2)	7 (3.4)
Any COVID-19 AE	0	0	3 (1.4)
Most frequent TEAEs (occurring in $\geq 10\%$ of patients)			
Back pain	108 (20.6)	111 (21.1)	11 (5.3)
Arthralgia	103 (19.7)	82 (15.6)	15 (7.2)
Pain in extremity	70 (13.4)	67 (12.7)	8 (3.8)
Fatigue	107 (20.4)	89 (16.9)	15 (7.2)
Bone pain	39 (7.4)	54 (10.2)	2 (1.0)
Rash	106 (20.2)	23 (4.4)	26 (12.5)
Pruritus	58 (11.1)	25 (4.7)	13 (6.3)
Constipation	59 (11.3)	58 (11.0)	6 (2.9)
Diarrhea	56 (10.7)	35 (6.6)	11 (5.3)
Hot flush	121 (23.1)	87 (16.5)	3 (1.4)
Hypertension	102 (19.5)	84 (15.9)	13 (6.3)
Weight increased	55 (10.5)	92 (17.5)	7 (3.4)
Anemia	69 (13.2)	72 (13.7)	13 (6.3)
Treatment-related TEAEs occurring in $\geq 5\%$ of patients ^b			
Patients with ≥ 1 treatment-related TEAE	319 (60.9)	221 (41.9)	93 (44.7)
Rash	92 (17.6)	12 (2.3)	23 (11.1)
Pruritus	43 (8.2)	13 (2.5)	8 (3.8)

Fatigue		71 (13.5)		46 (8.7)		14 (6.7)
Hot flush		67 (12.8)		52 (9.9)		3 (1.4)
Hypertension		28 (5.3)		21 (4.0)		5 (2.4)
AEs of interest by group term and grades ^c						
	All grades	Grade ≥ 3	All grades	Grade ≥ 3	All grades	Grade ≥ 3
Skin rash, ^d	153 (29.2)	33 (6.3)	49 (9.3)	5 (0.9)	45 (21.6)	8 (3.8)
Fracture ^e	54 (10.3)	18 (3.4)	26 (4.9)	4 (0.8)	5 (2.4)	0
Fall	49 (9.4)	7 (1.3)	37 (7.0)	5 (0.9)	8 (3.8)	0
Ischemic heart disease ^f	31 (5.9)	16 (3.1)	11 (2.1)	4 (0.8)	1 (0.5)	1 (0.5)
Ischemic cerebrovascular disorder ^g	13 (2.5)	8 (1.6)	8 (1.5)	1 (0.2)	5 (2.4)	5 (2.4)
Seizure ^h	3 (0.6)	1 (0.2)	2 (0.4)	0	0	0

^aShown are adverse events (AEs) of any cause, unless otherwise noted, that occurred from the time of the first dose of the trial intervention through 30 days after the last dose. For each category, patients with multiple events were counted only once. AEs were graded according to National Cancer Institute Common Terminology Criteria for Adverse Events, version 4.0.3. One patient who was assigned to the apalutamide group withdrew consent before treatment.

^bExcludes grade 5 events.

^cThe worst toxicity grade is included. Patients with missing toxicity grade were counted in the all-grade column.

^dSkin rash was a grouped term including rash, maculo-papular rash, conjunctivitis, dermatitis, stomatitis, pruritic rash, urticaria, papular rash, skin exfoliation, blister, mouth ulceration, drug eruption, erythema multiforme, exfoliative rash, toxic skin eruption, papule, skin reaction, butterfly rash, generalized exfoliative dermatitis, genital rash, erythematous rash, macular rash, systemic lupus erythematosus rash, oral mucosal blistering, follicular rash, pustular rash, and vesicular rash.

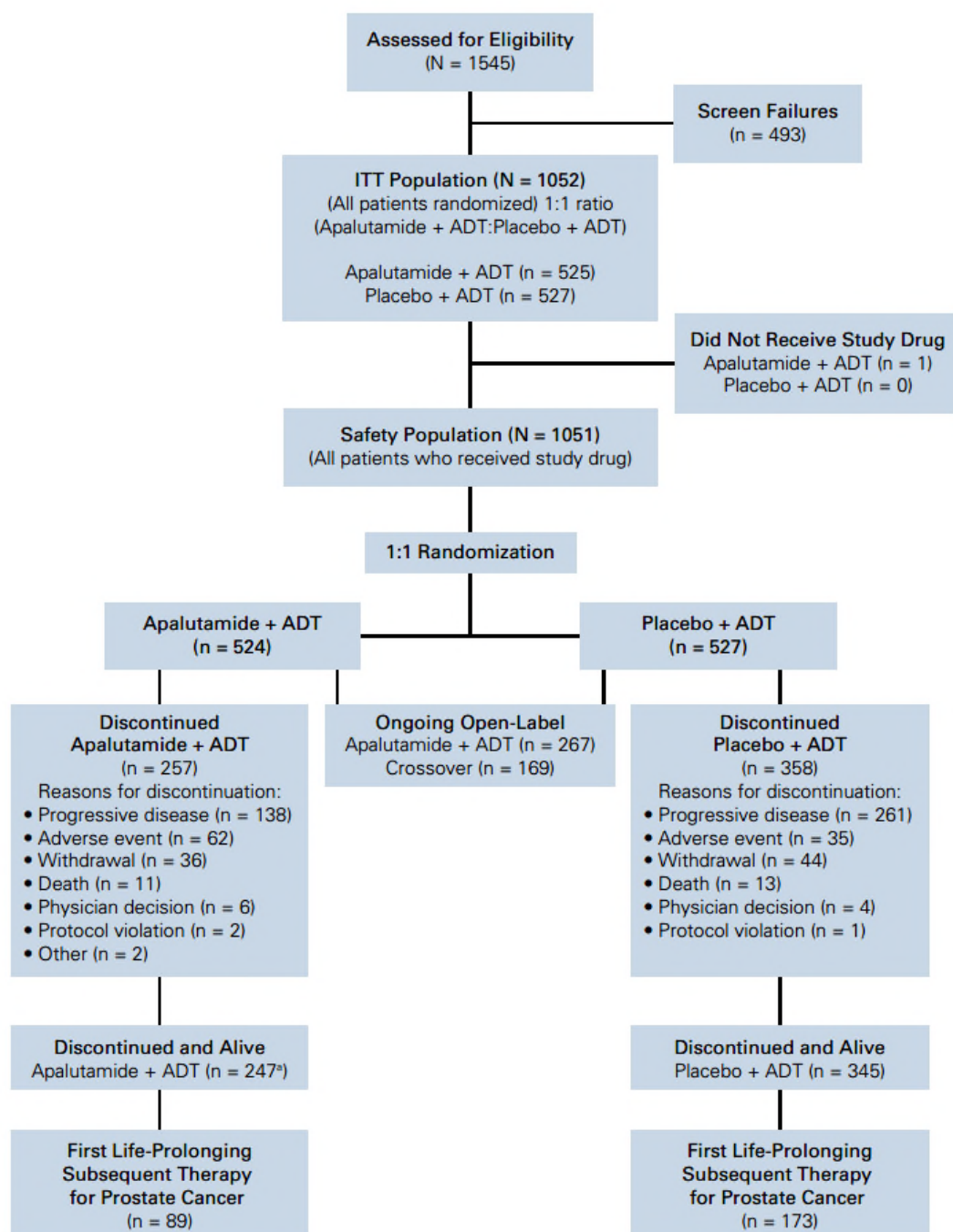
^eFracture was a grouped term including rib fracture, spinal compression fracture, hand fracture, femoral neck fracture, foot fracture, femur fracture, thoracic vertebral fracture, traumatic fracture, upper limb fracture, wrist fracture, ankle fracture, fracture, hip fracture, spinal fracture, radius fracture, acetabulum fracture, fracture pain, clavicle fracture, comminuted fracture, compression fracture, forearm fracture, humerus fracture, patella fracture, pelvic fracture, sternal fracture, stress fracture, ulna fracture, fibula fracture, lower limb fracture, skull fracture, and tibia fracture.

^fIschemic heart disease was a group term including angina pectoris, myocardial infarction, acute myocardial infarction, coronary artery stenosis, coronary artery arteriosclerosis, myocardial ischaemia, coronary artery disease, coronary artery occlusion, acute coronary syndrome, abnormal cardiac stress test, ischaemic cardiomyopathy, unstable angina, and increased troponin.^gIschemic cardiovascular disorder was a group term including cerebrovascular accident, transient ischemic attack, ischemic stroke,

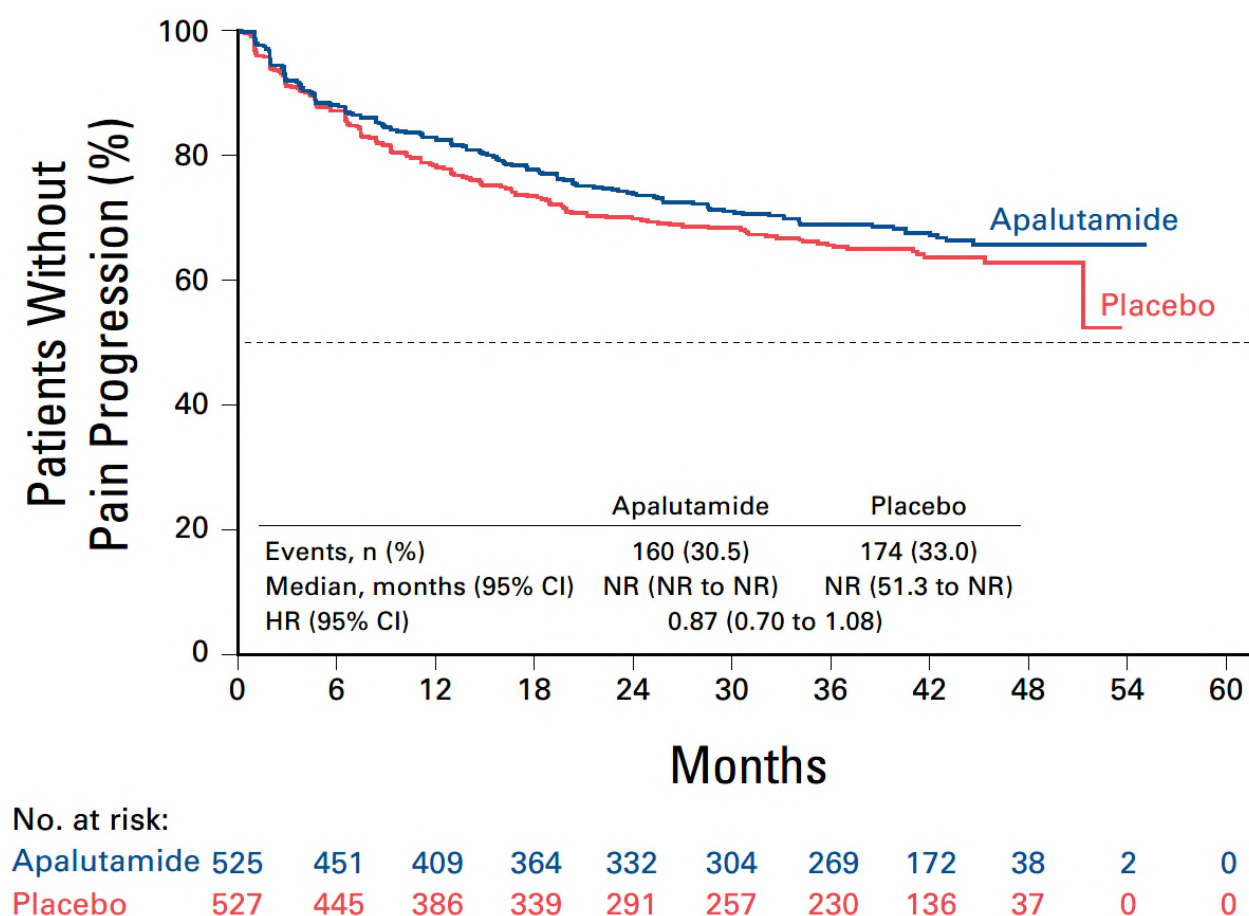
cerebrovascular disorder, lacunar infarction, cerebral ischemia, hemiplegia, vascular encephalopathy, carotid artery stenosis, and carotid arteriosclerosis.

^hSeizure was a group term including seizure and tongue biting.

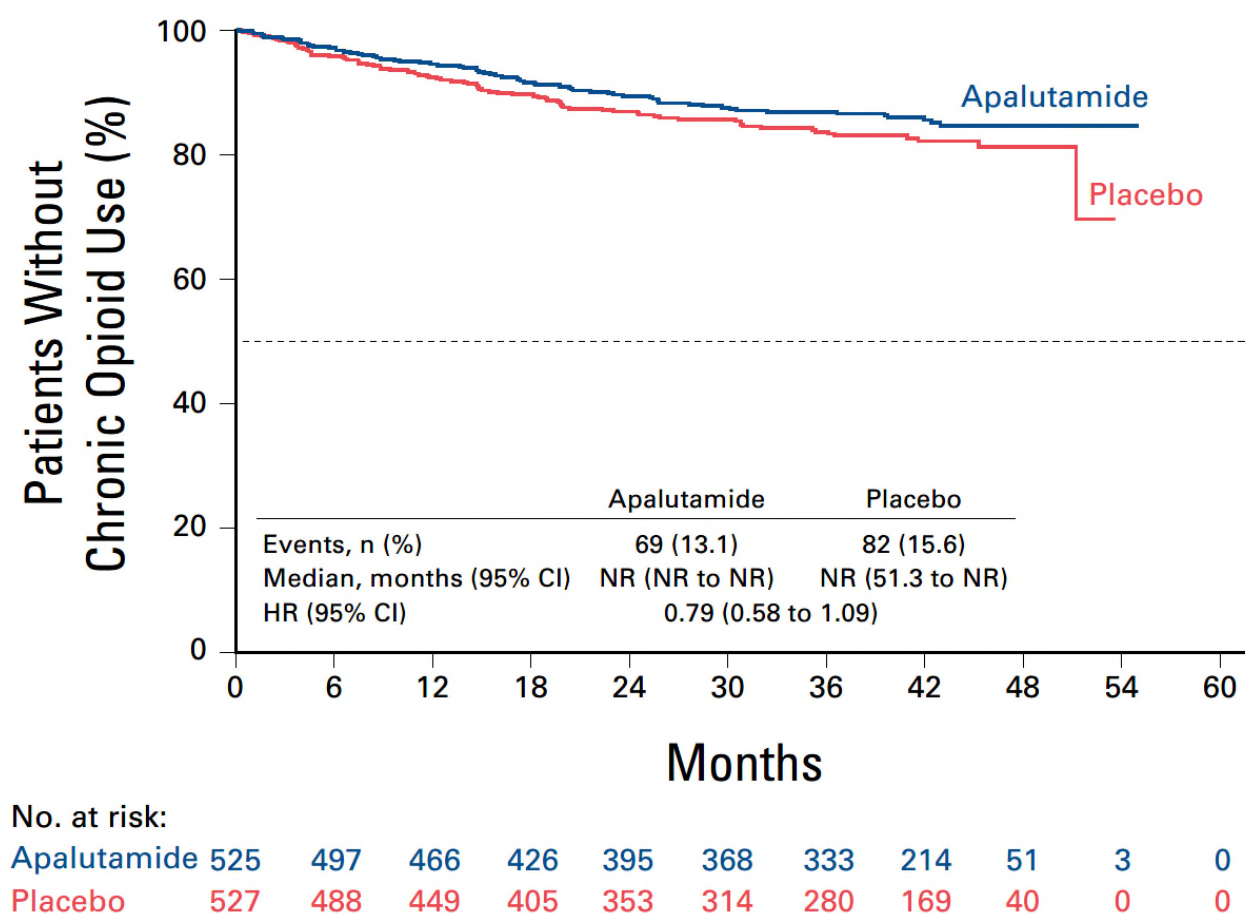
Abbreviations: ADT, androgen deprivation therapy; TEAE, treatment-emergent AE.

SUPPLEMENTAL FIG. S1. CONSORT diagram.^aIncludes 1 patient who did not receive apalutamide.

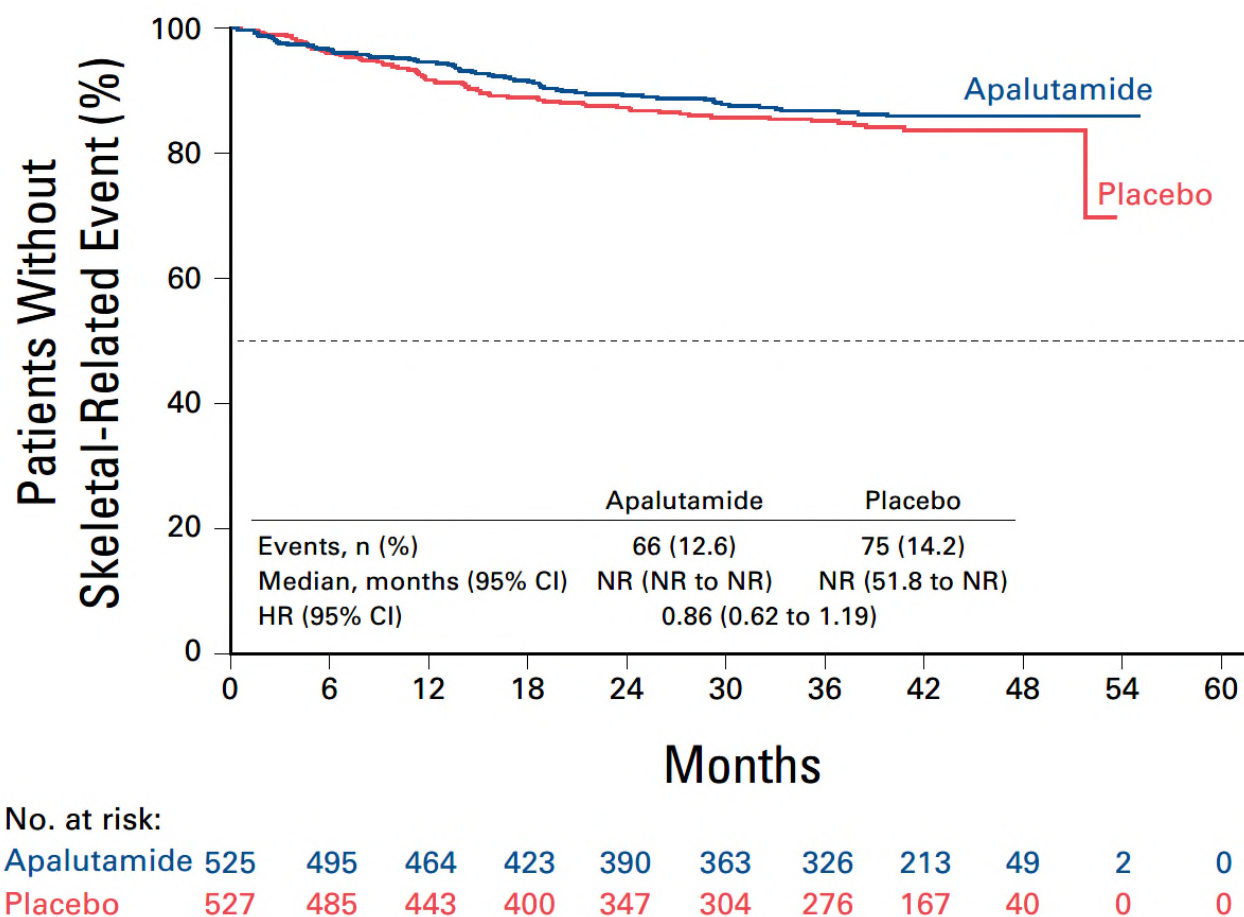
SUPPLEMENTAL FIG. S2. Kaplan-Meier estimate of time to pain progression.



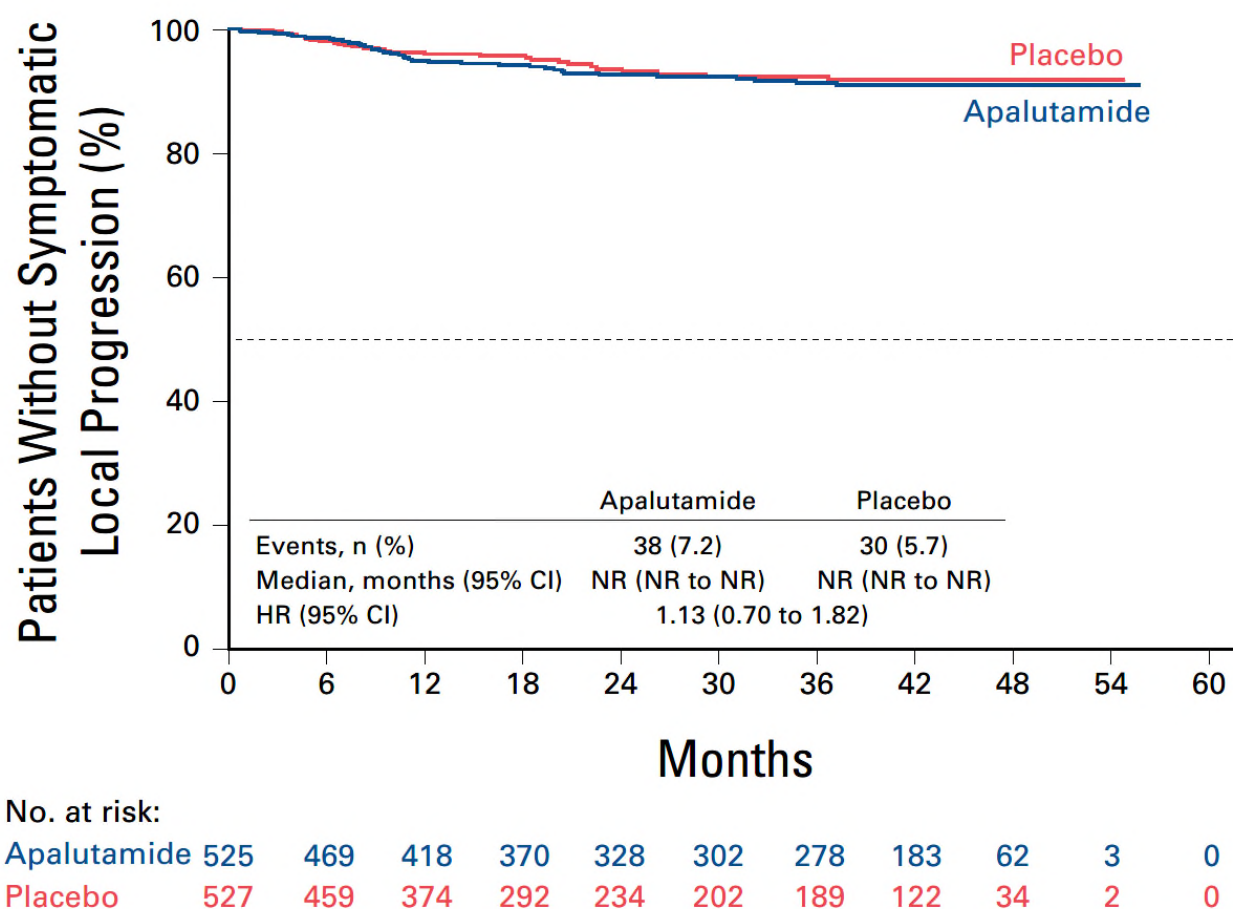
SUPPLEMENTAL FIG. S3. Kaplan-Meier estimate of time to chronic opioid use.



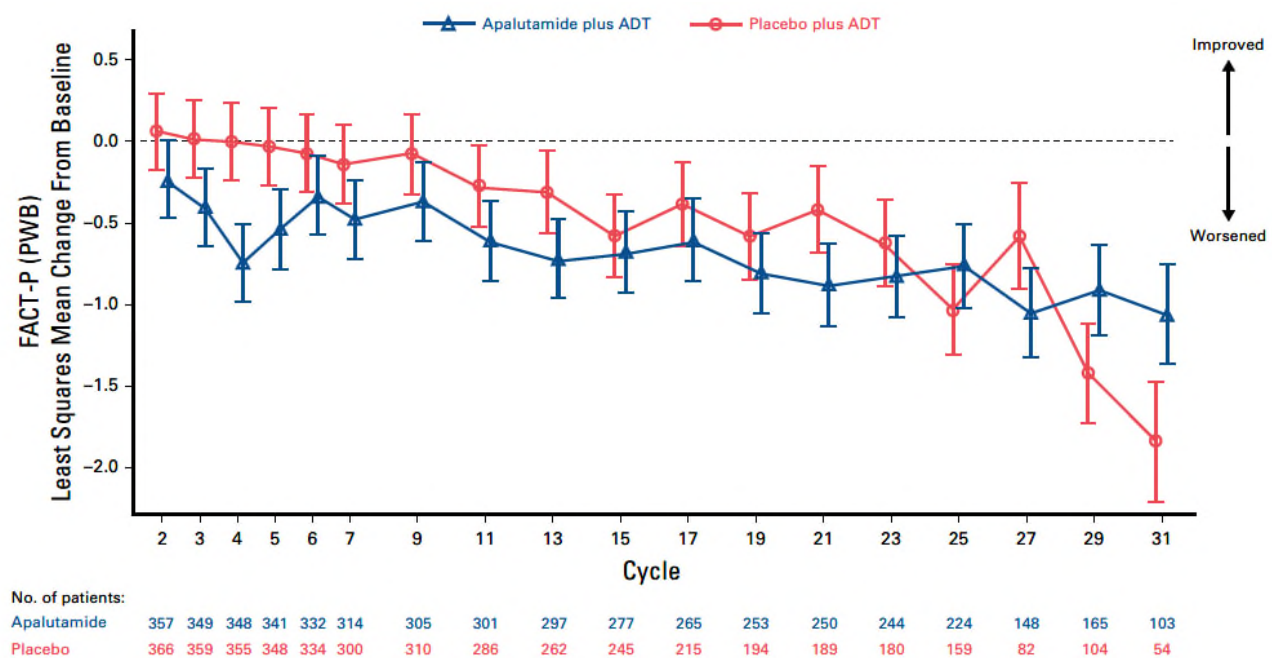
SUPPLEMENTAL FIG. S4. Kaplan-Meier estimate of time to skeletal-related event.



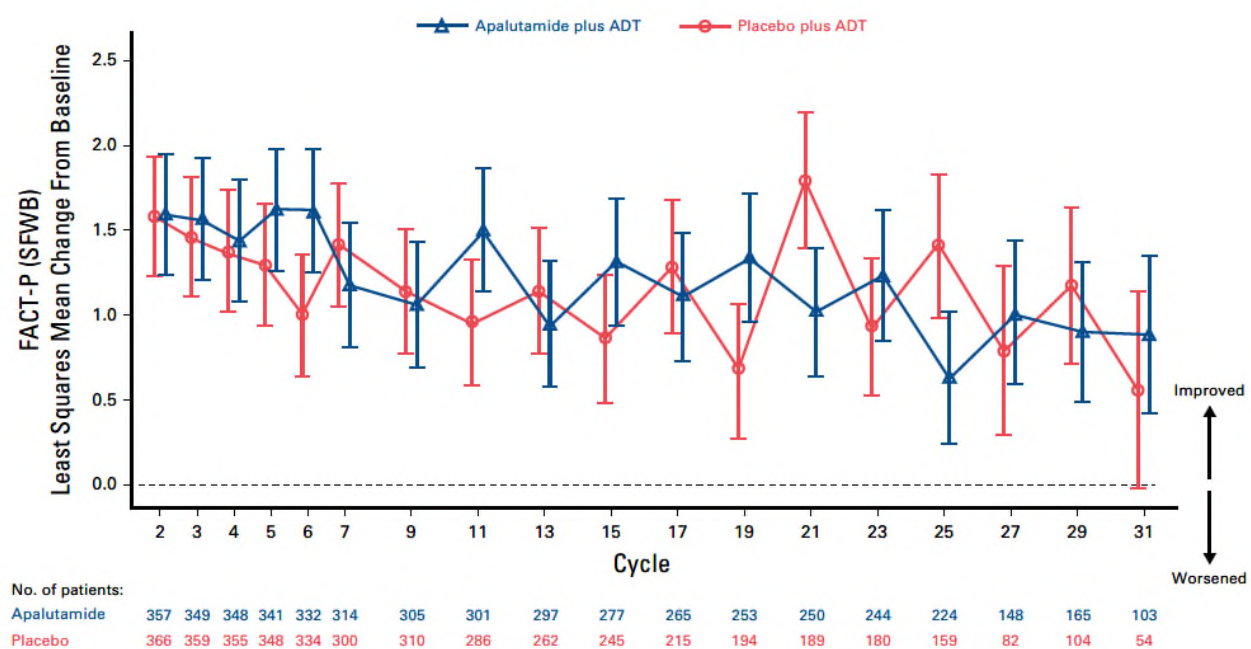
SUPPLEMENTAL FIG. S5. Kaplan-Meier estimate of time to symptomatic local progression.



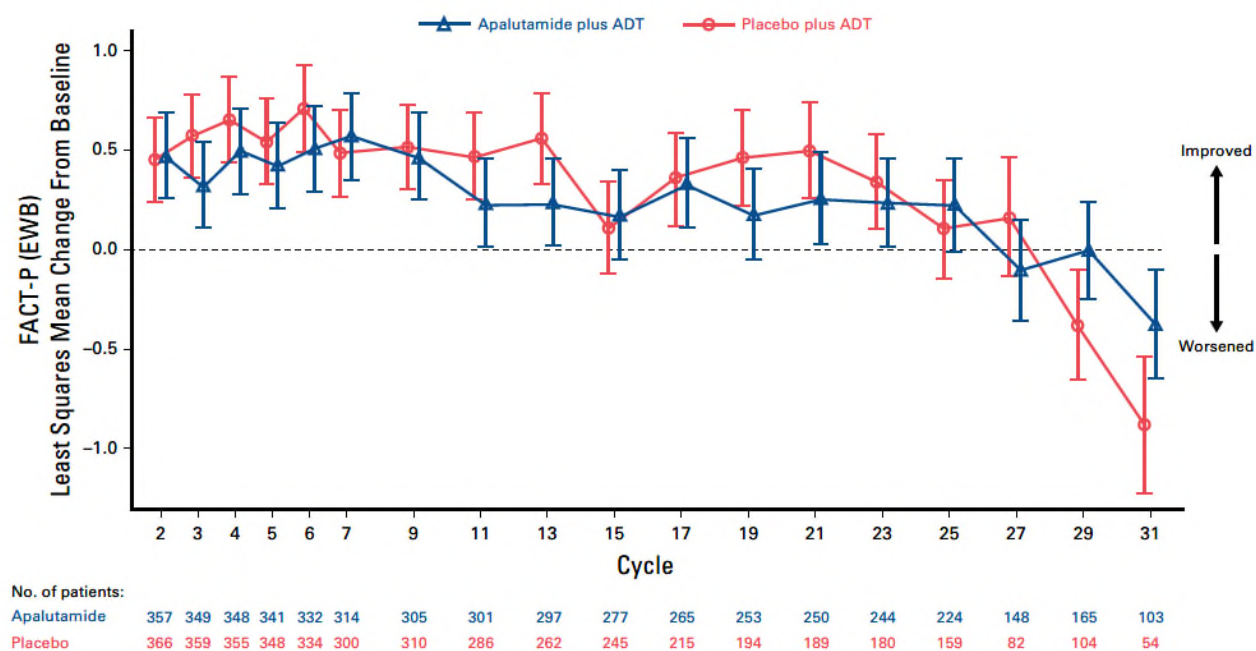
SUPPLEMENTAL FIG. S6. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Physical Well-Being subscale score.



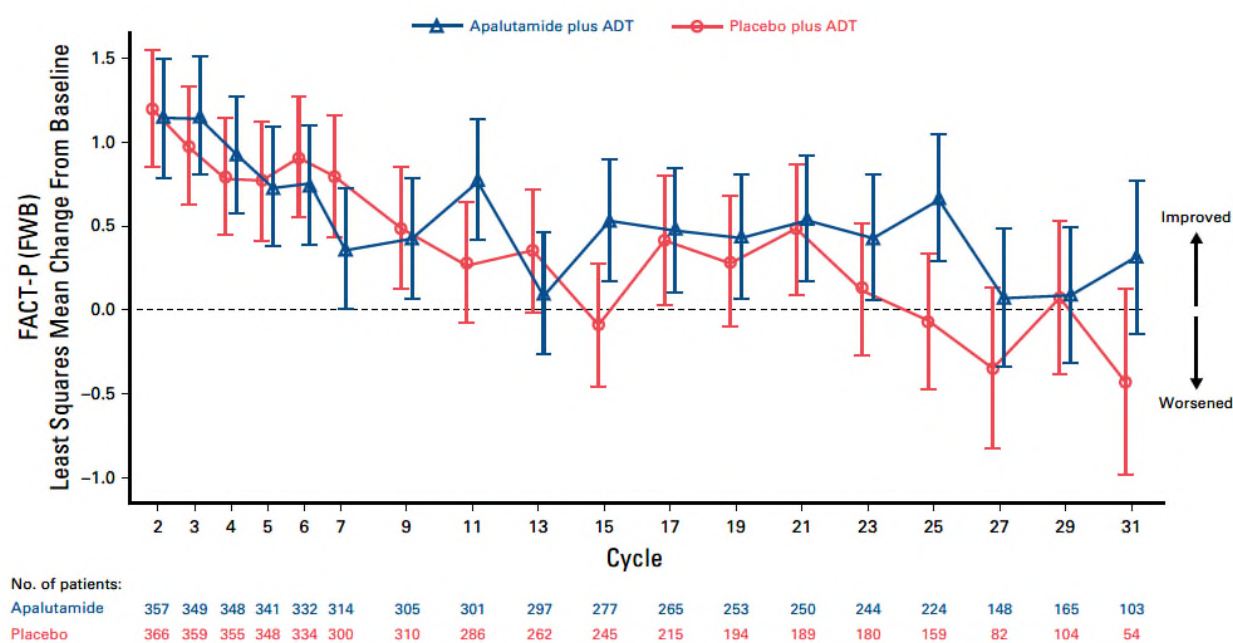
SUPPLEMENTAL FIG. S7. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Social/Family Well-Being subscale score.



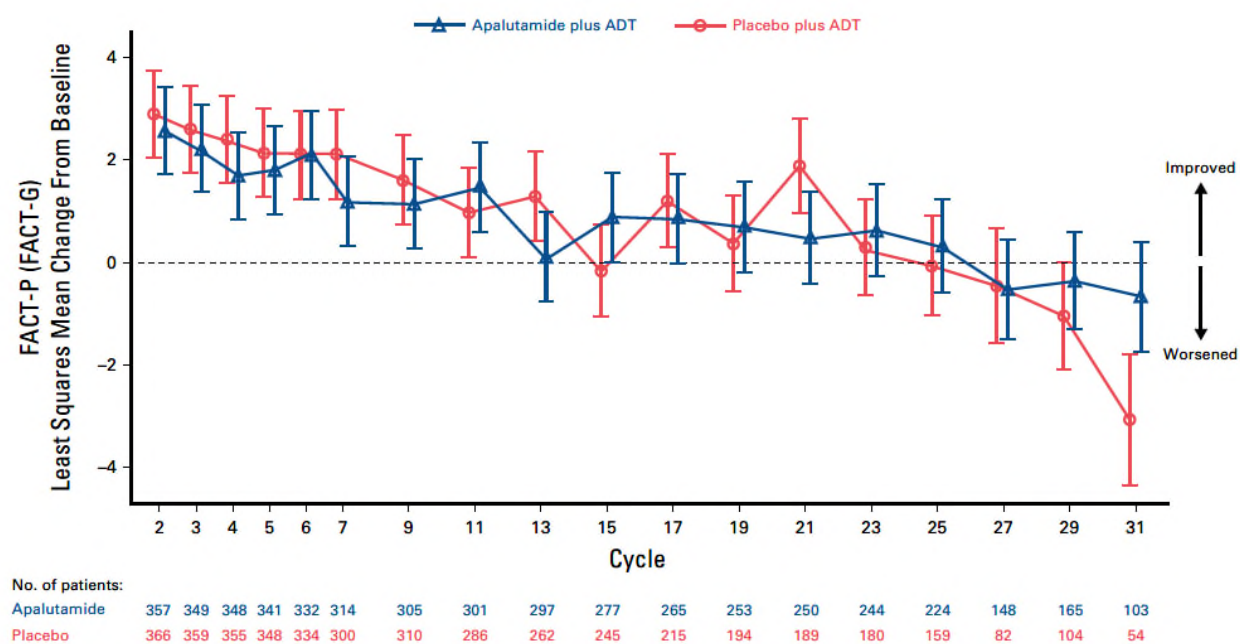
SUPPLEMENTAL FIG. S8. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Emotional Well-Being subscale score.



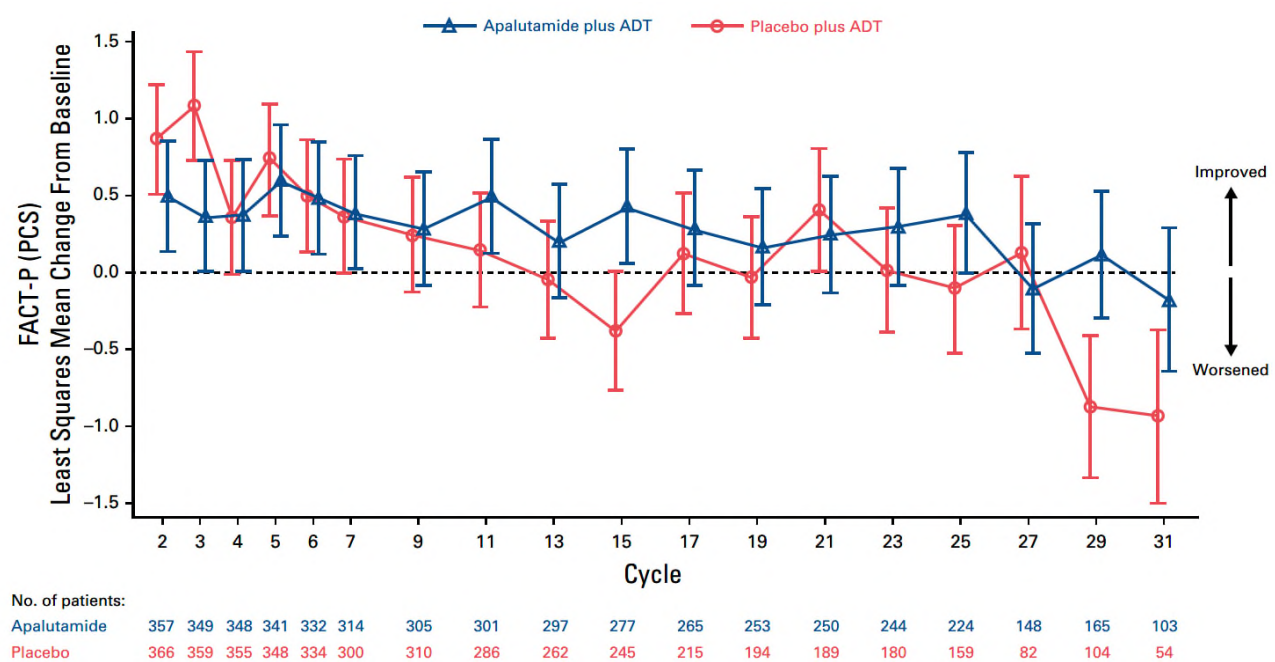
SUPPLEMENTAL FIG. S9. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Functional Well-Being subscale score.



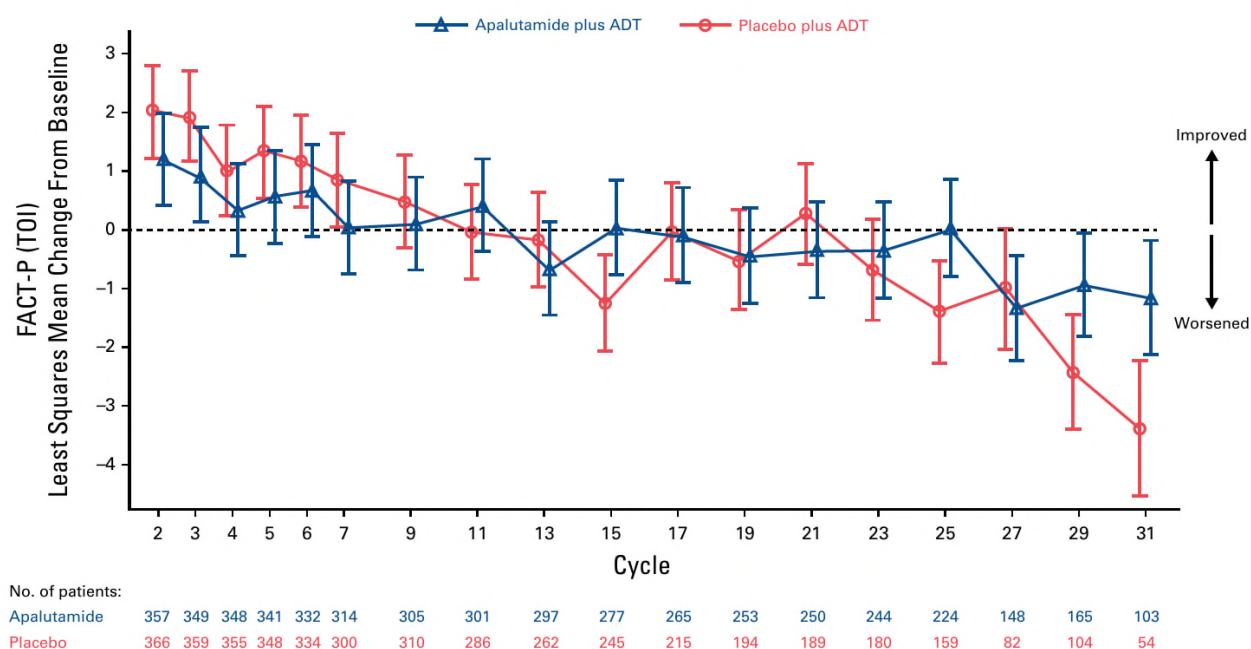
SUPPLEMENTAL FIG. S10. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-G subscale score.



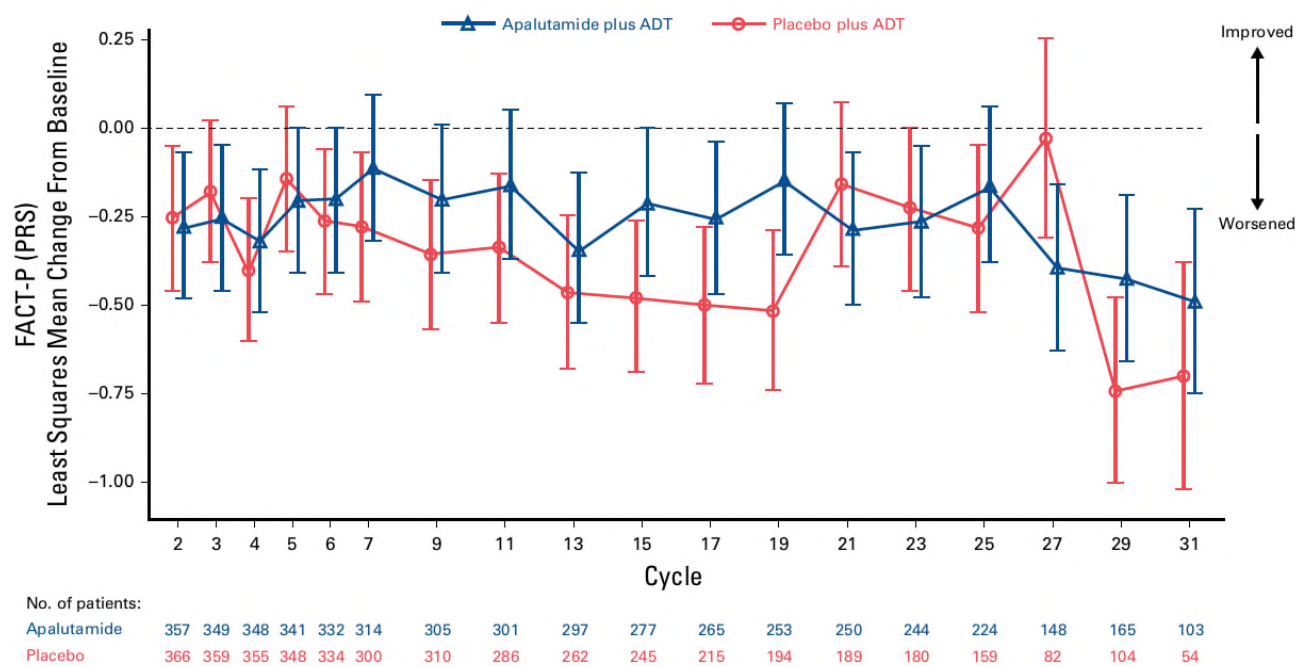
SUPPLEMENTAL FIG. S11. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Prostate Cancer Subscale score.



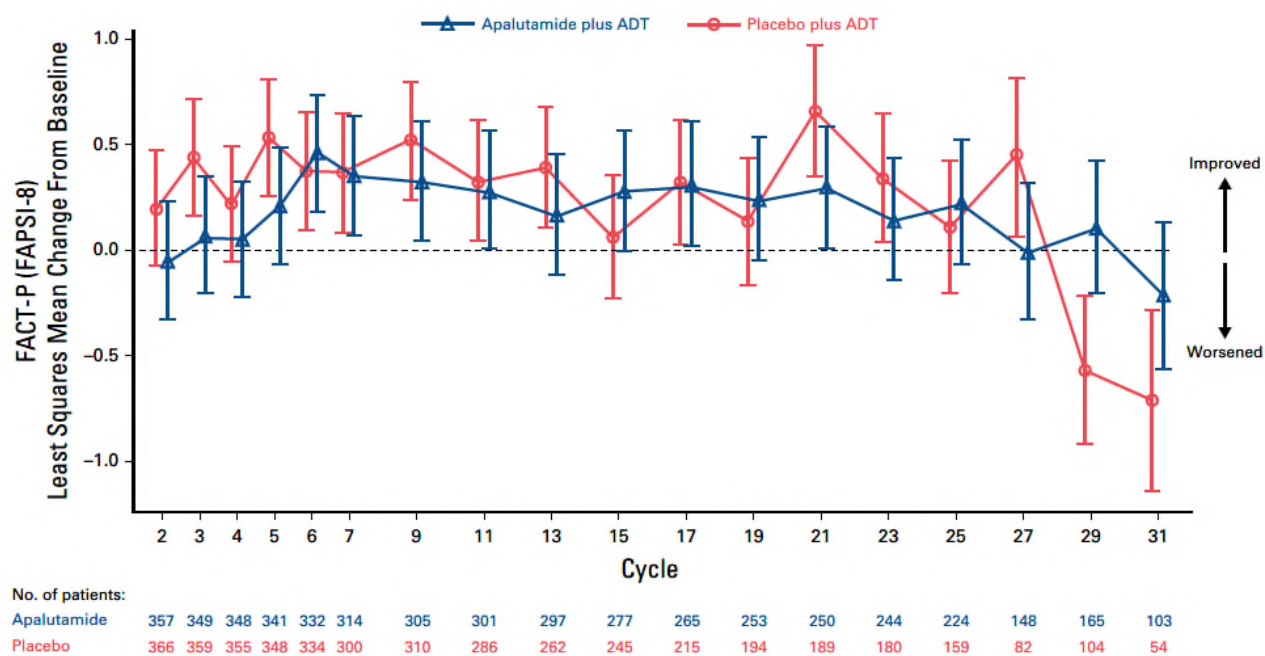
SUPPLEMENTAL FIG. S12. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Trial Outcome Index.

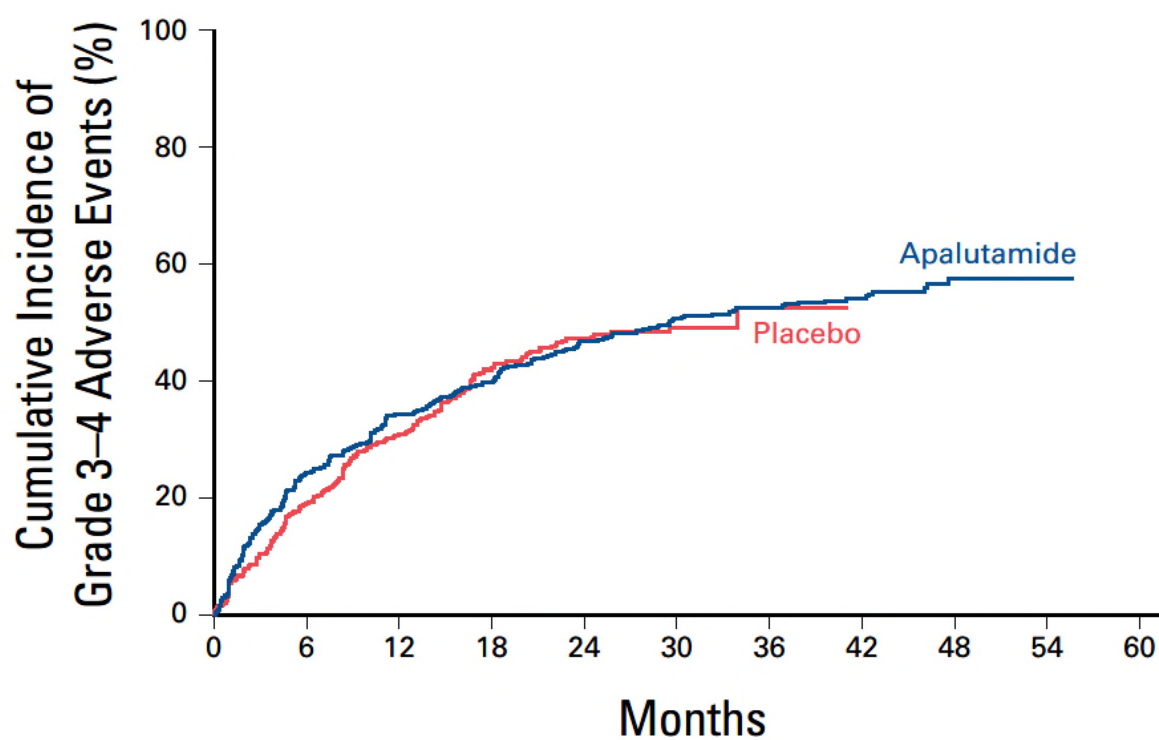


SUPPLEMENTAL FIG. S13. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT-P Pain-Related Subscale score.



SUPPLEMENTAL FIG. S14. Mixed-effects repeated-measures analyses for least squares mean change from baseline in FACT Advanced Prostate Symptom Index-8.

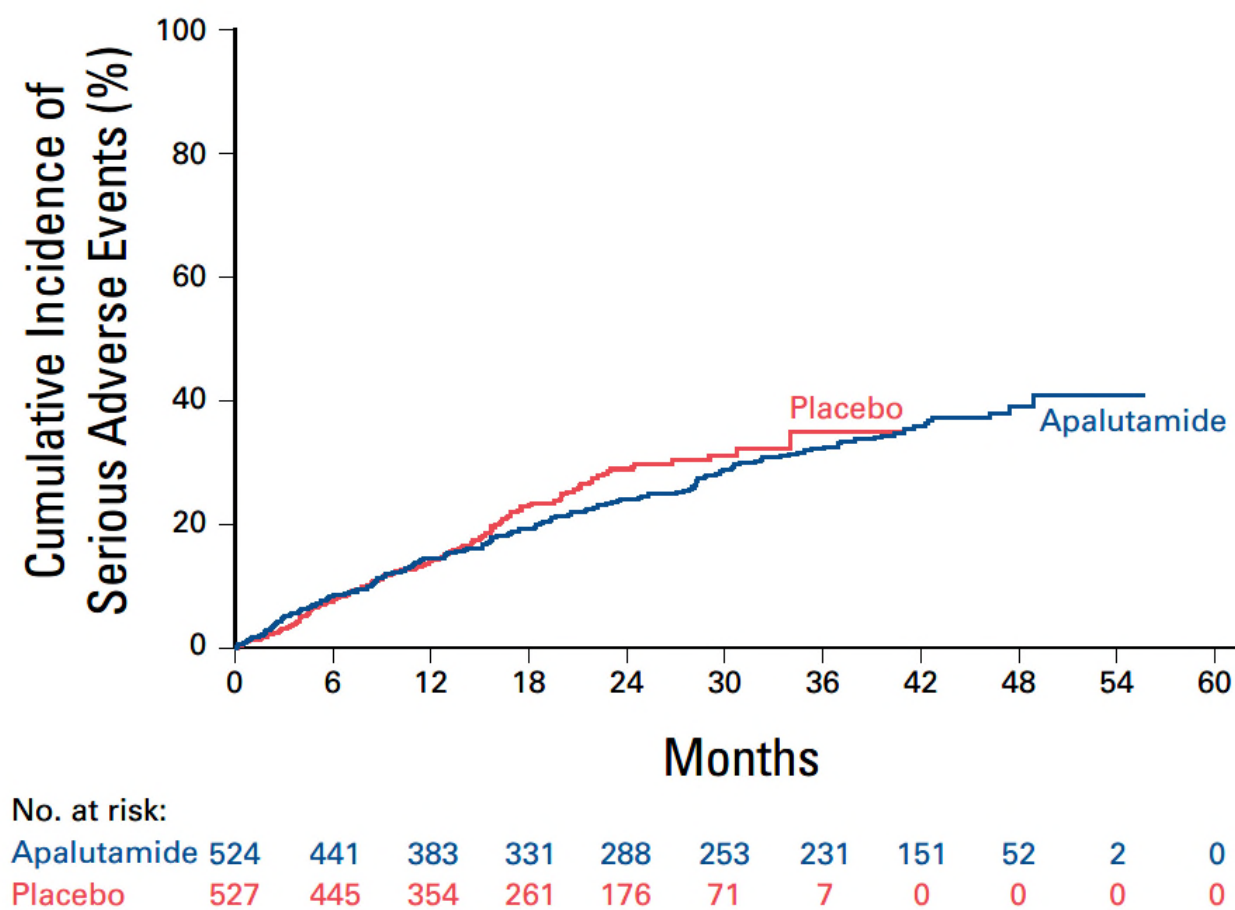


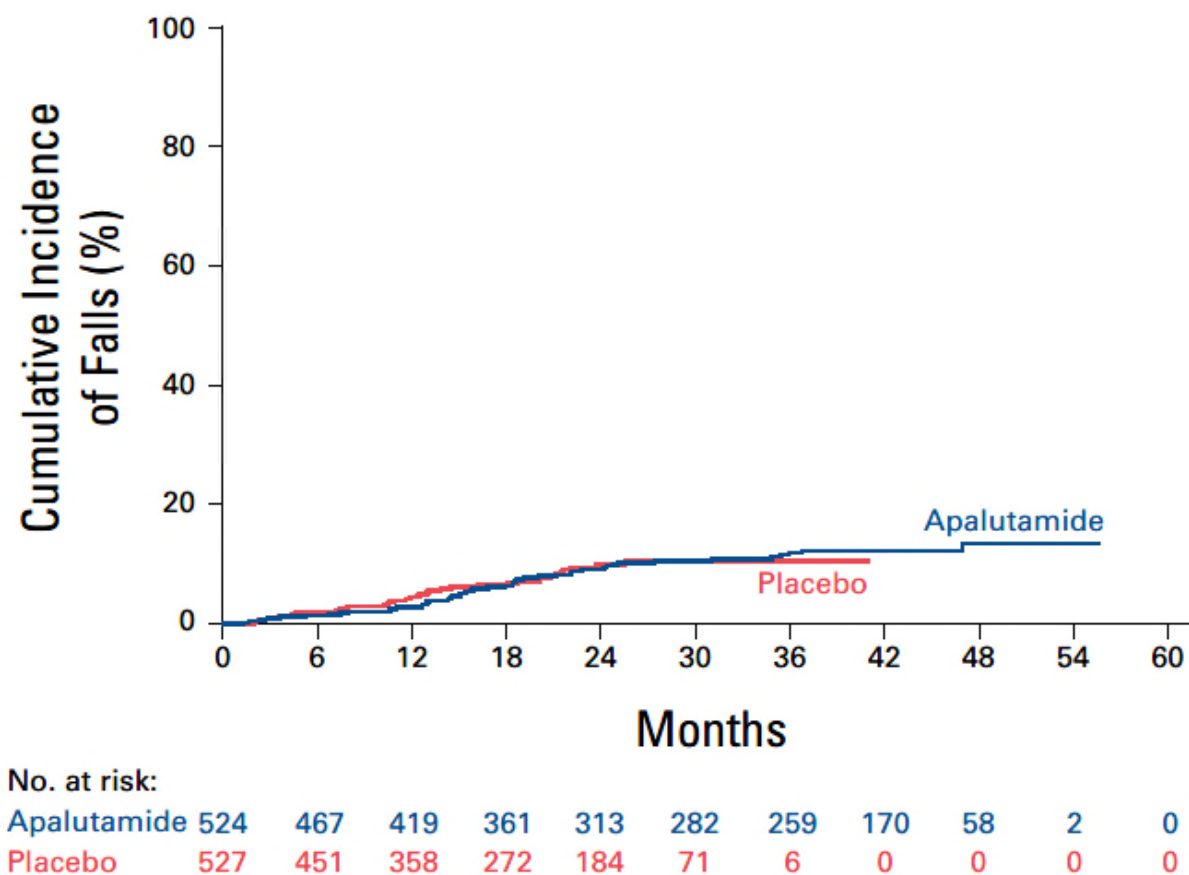
SUPPLEMENTAL FIG. S15. Cumulative incidence of first grade 3 to 4 adverse event.

No. at risk:

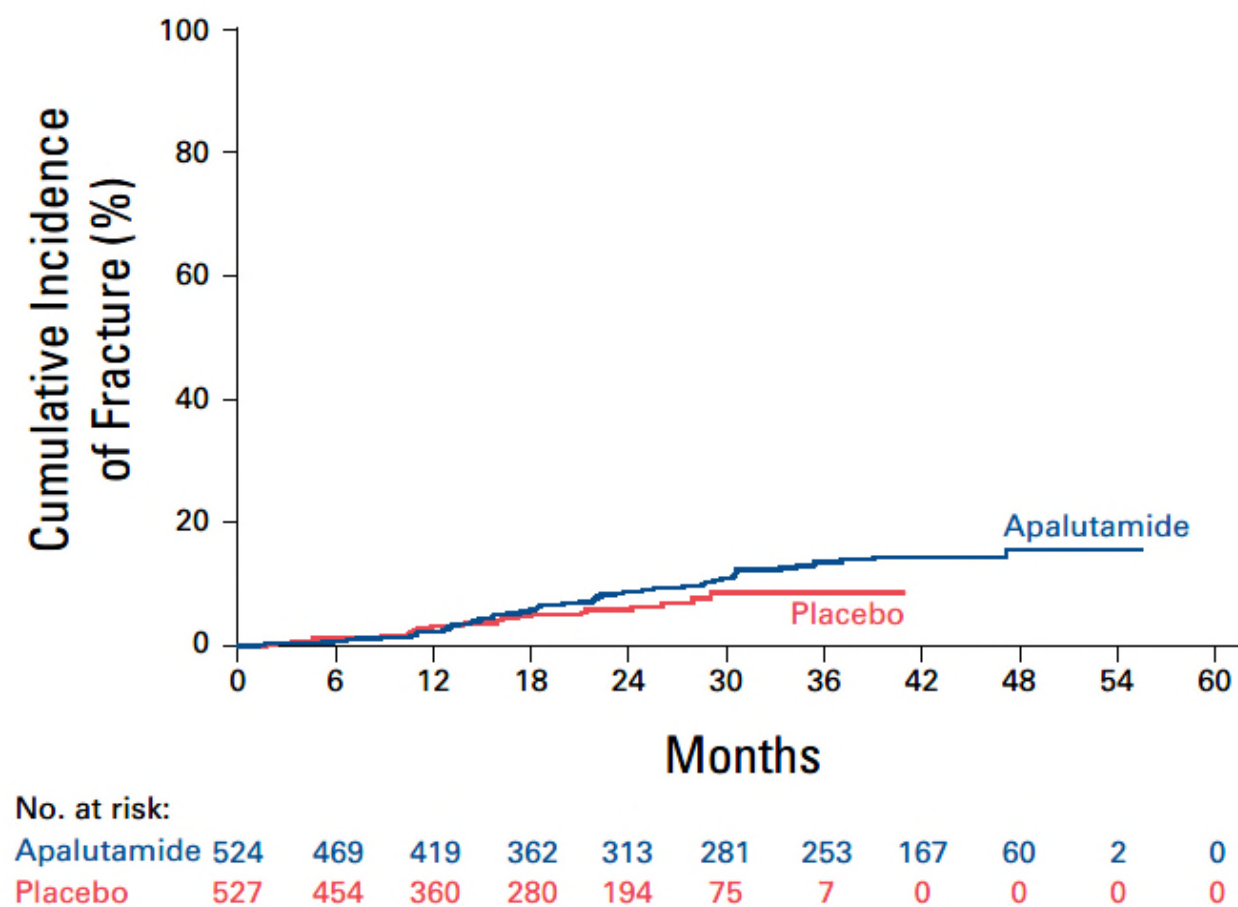
Apalutamide	524	373	305	258	213	185	171	119	43	2	0
Placebo	527	398	296	212	146	55	5	0	0	0	0

SUPPLEMENTAL FIG. S16. Cumulative incidence of first serious adverse event.

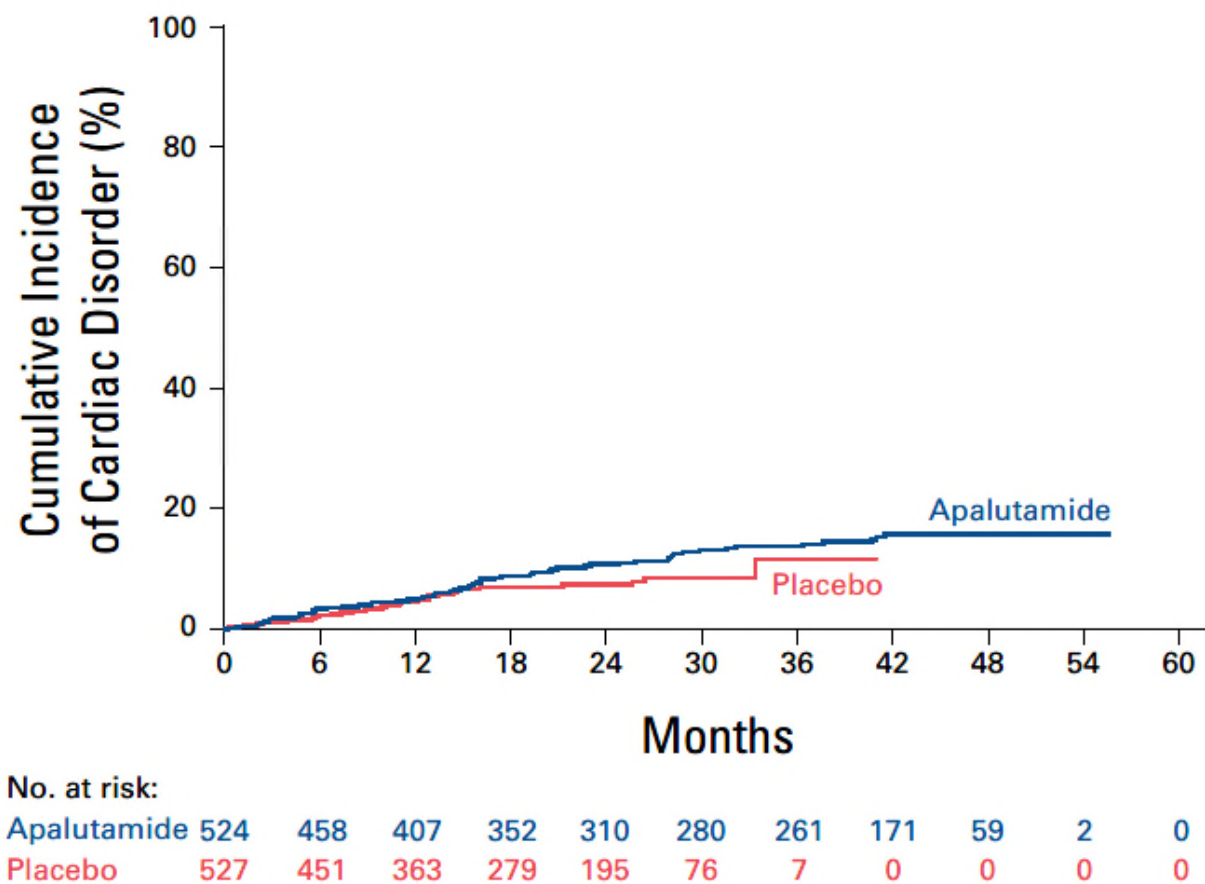


SUPPLEMENTAL FIG. S17. Cumulative incidence of first any-grade, treatment-emergent fall.

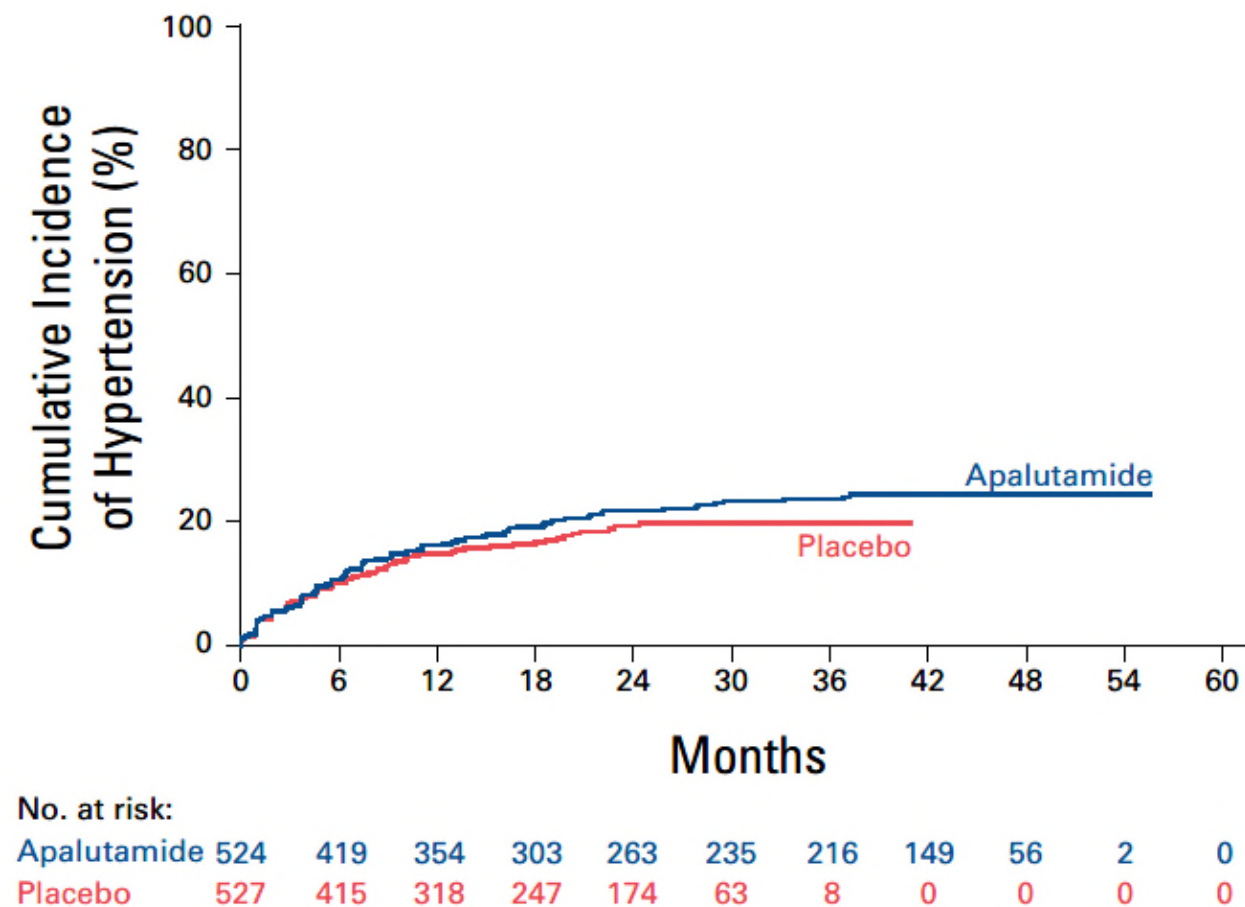
SUPPLEMENTAL FIG. S18. Cumulative incidence of first any-grade, treatment-emergent fracture.



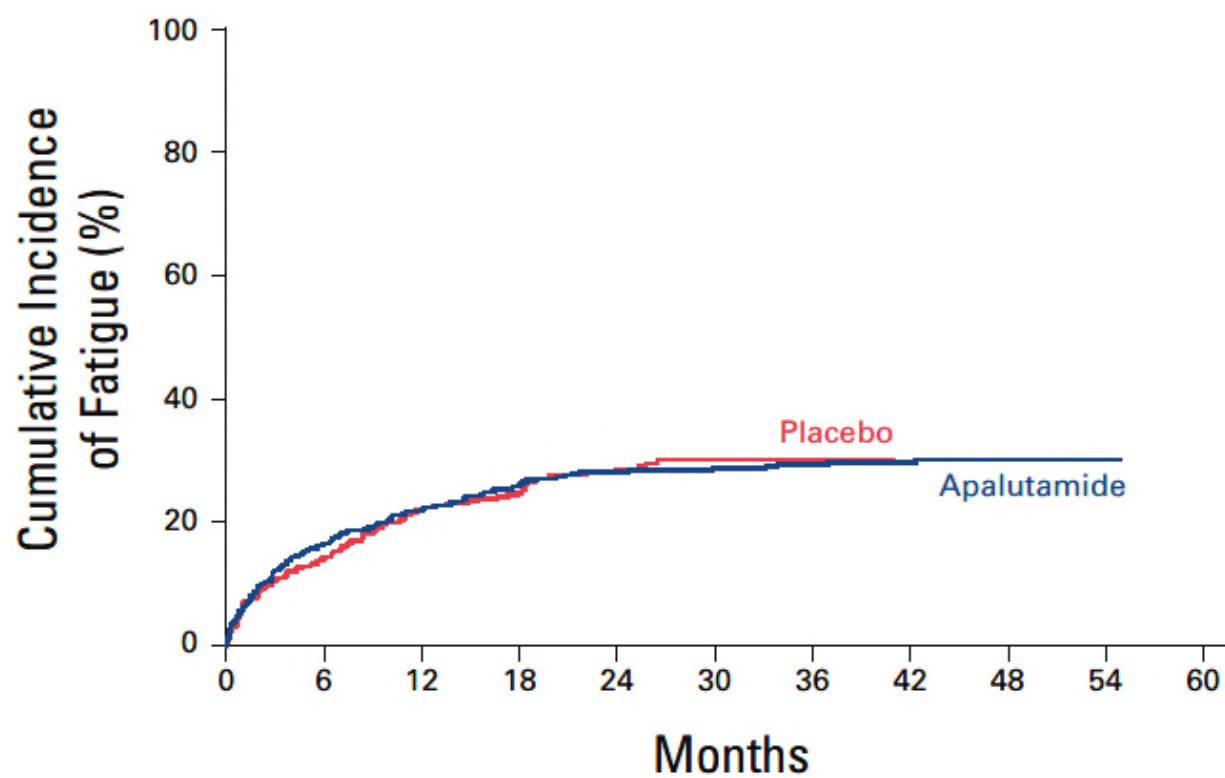
SUPPLEMENTAL FIG. S19. Cumulative incidence of first any-grade, treatment-emergent cardiac disorder event.



SUPPLEMENTAL FIG. S20. Cumulative incidence of first any-grade, treatment-emergent hypertension event.



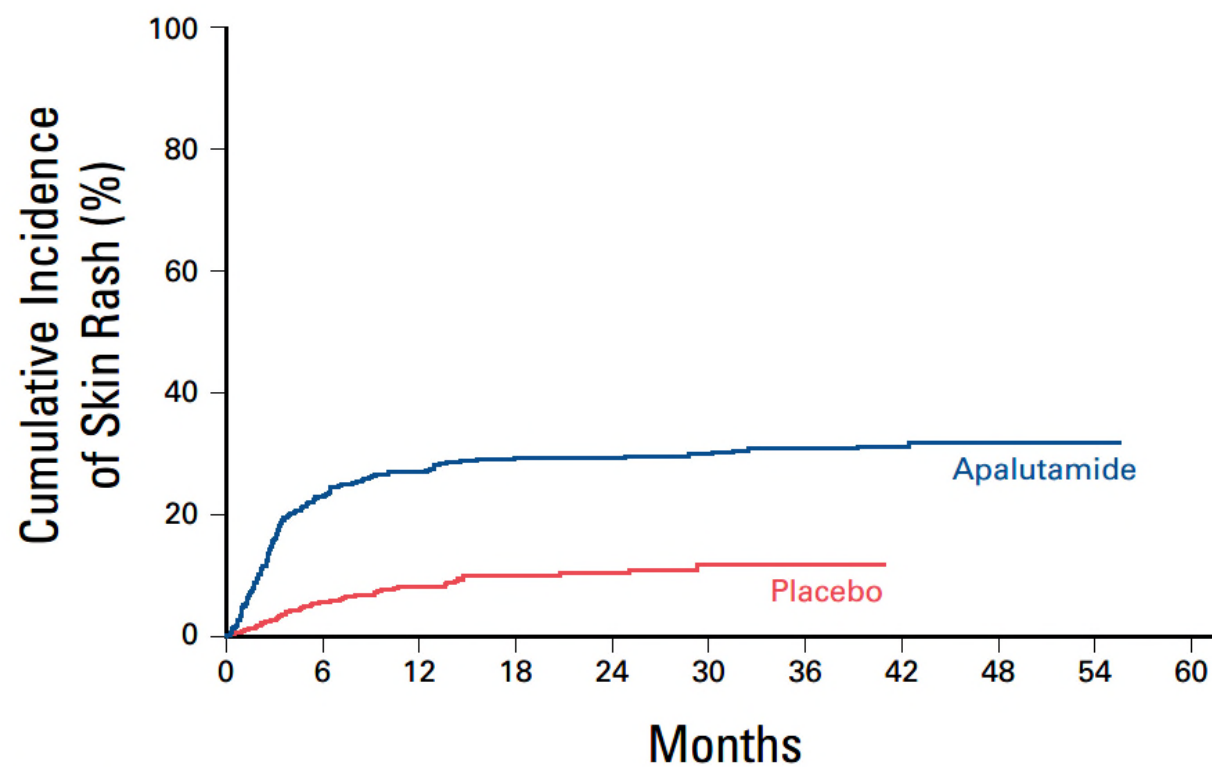
SUPPLEMENTAL FIG. S21. Cumulative incidence of first any-grade, treatment-emergent fatigue event.



No. at risk:

Apalutamide	524	401	346	295	260	236	217	143	48	2	0
Placebo	527	404	305	235	154	60	5	0	0	0	0

SUPPLEMENTAL FIG. S22. Cumulative incidence of first any-grade, treatment-emergent skin rash event.



No. at risk:

Apalutamide	524	368	317	275	246	223	201	139	47	3	0
Placebo	527	433	340	264	189	73	7	0	0	0	0